The U.S. Clinical-Trial Investigator Supply

A preliminary descriptive analysis of FDA BMIS Form 1572 filings, benchmarked against industry trials

Strict persons · 2009–2025 · generated 2026-11-06

PRELIMINARY ANALYSIS ONLY — NOT FOR DECISION-MAKING

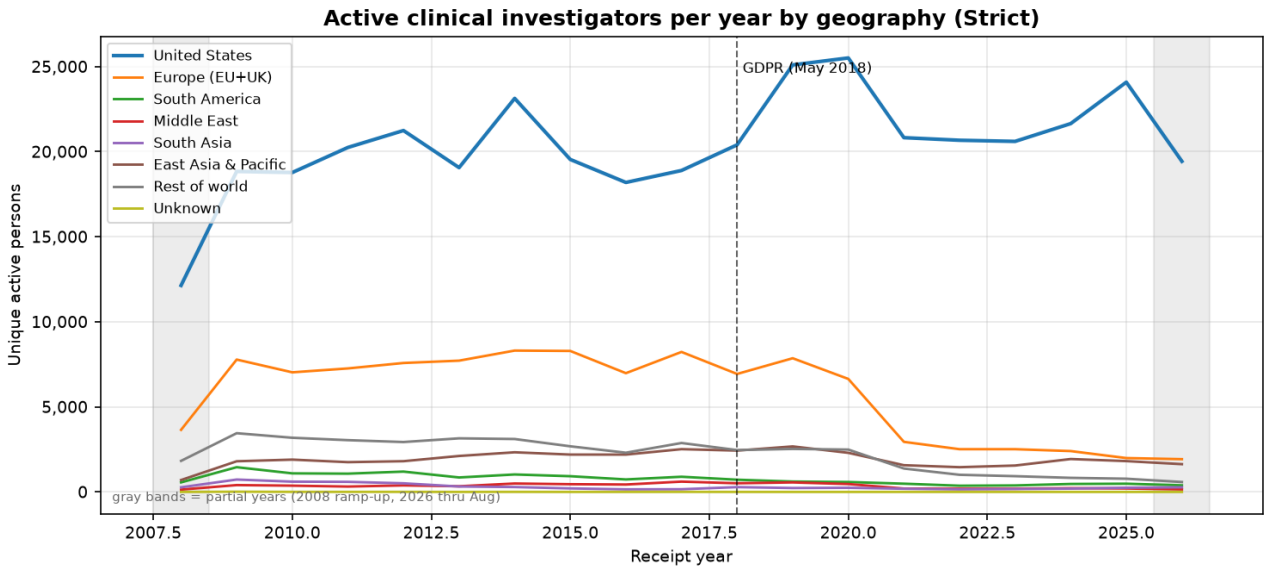
FDA BMIS (Form 1572, thru Aug 2026) · clinicaltrials.gov API v2 · github.com/henryweimd/bmis1572

What this is (and isn’t)

- Supply = clinical investigators (CIs) named on Form 1572 filings in FDA’s BMIS file.
- Persons are name+site clusters: Strict (headline), Merged, Raw bound the identity call.
- Active in year = ≥1 receipt; headline window 2009–2025.
- Demand = industry-led interventional trials (clinicaltrials.gov API), starts + running.
- Benchmarks are year-level ratios — no record-level CI↔trial linkage exists.

BMIS covers CDER IND drugs/biologics only; FDA states it is not comprehensive.
2008 is file ramp-up, 2026 partial (thru Aug); both excluded from growth stats.

Headline: global flat-to-down, U.S. up

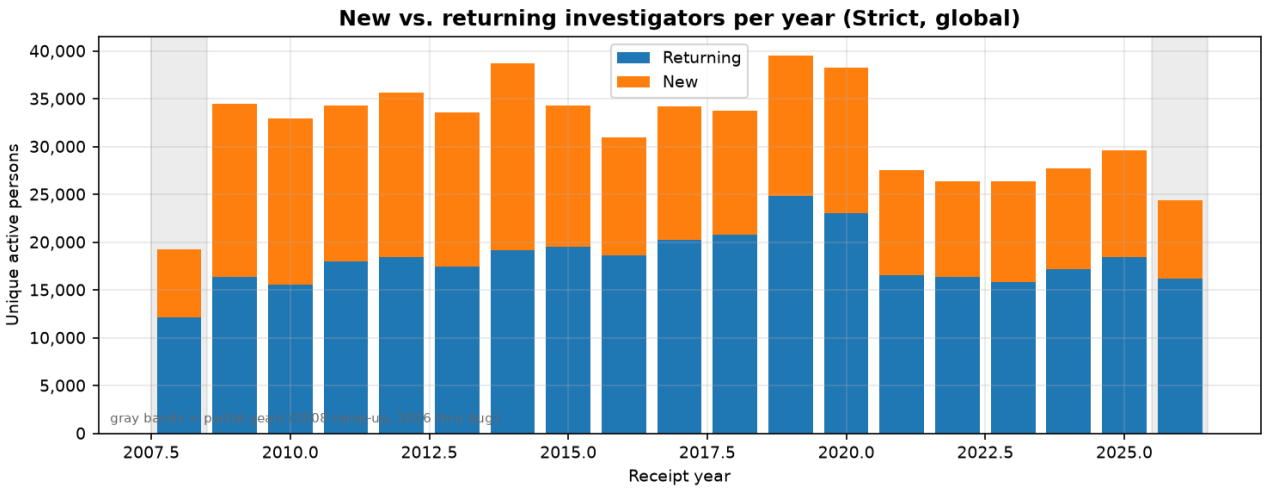


DEFINITIONS: Strict person: rows sharing identical normalized last/first/middle name + site + street + city + state + ZIP + country (exact-duplicate rows collapsed first). Conservative: one human at two address spellings counts twice. Active in year: person has >=1 BMIS receipt in that calendar year. New: first receipt ever falls in that year (2008-09 inflated: file starts Oct 2008). Returning: active and first seen earlier. BMIS covers CDER IND drug/biologic investigators whose 1572/1571 was forwarded to FDA since Oct 2008. FDA: not a comprehensive list (1572 goes to the sponsor; forwarding is voluntary-ish). No devices, no CBER, no non-IND research.

- Global active CIs: 34,468 → 29,596 (-14.1%).
- U.S. active CIs: 18,838 → 24,084 (+27.8%).
- Ex-U.S. decline concentrates in 2020–21; no EU break at GDPR (May 2018).

Strict persons; receipt-year activity.

Fewer newcomers; a stable returning core

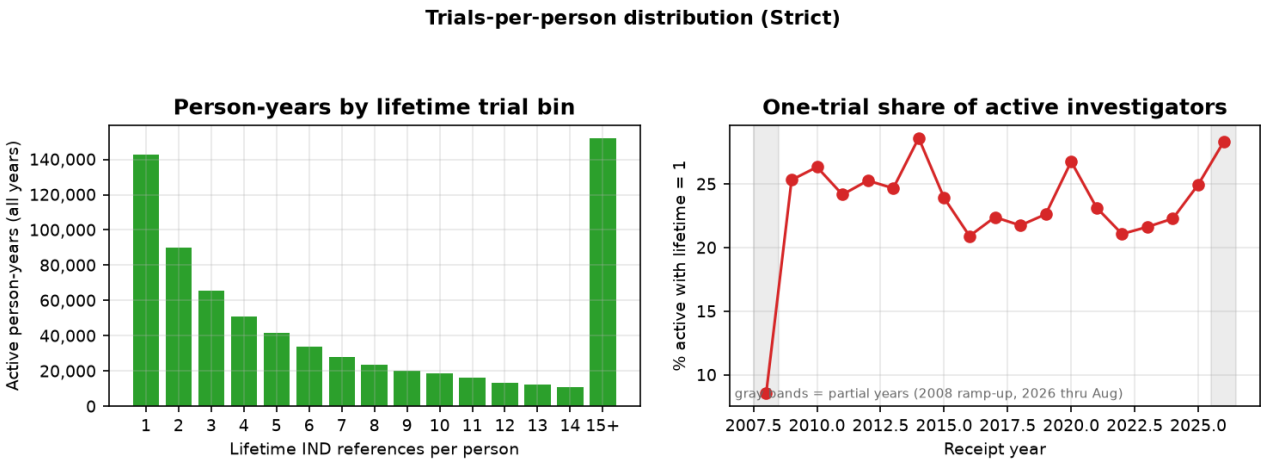


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- 38% of 2025 actives are first-timers; the rest return.
- Entry is dominated by one-and-done dynamics (next).

New = first receipt ever in that year.

Half of investigators are one-and-done

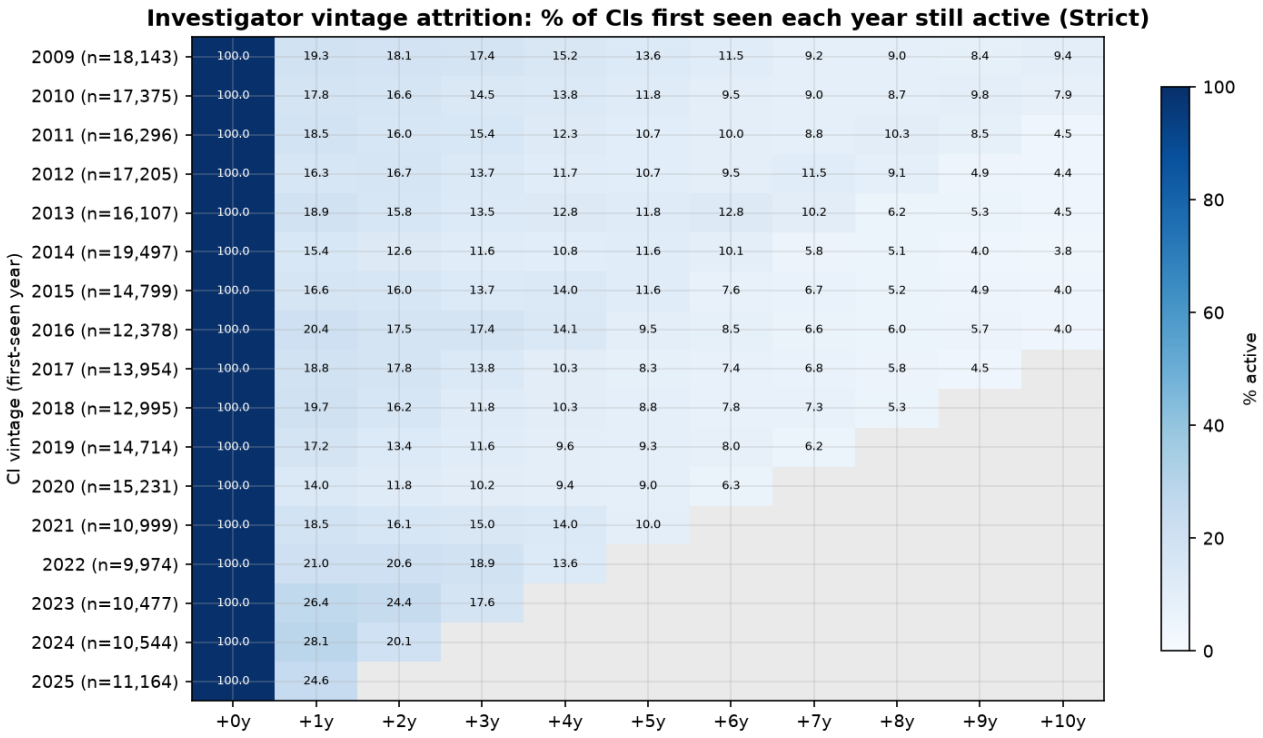


DEFINITIONS: Trials per person: collapsed BMIS rows per person-cluster. One row = one IND submission reference, a proxy for trials, not confirmed distinct trials. One-trial (one-and-done): person-cluster linked to exactly one IND reference across the full 1980-2026 file span.

- 49.3% of persons have exactly one lifetime IND reference.
- Median 2, mean 3.85 trials per investigator.
- A thin right tail of hyper-active CIs carries the top bins.

One row = one IND reference: a proxy, not confirmed distinct trials.

Vintage attrition: steep, then flat

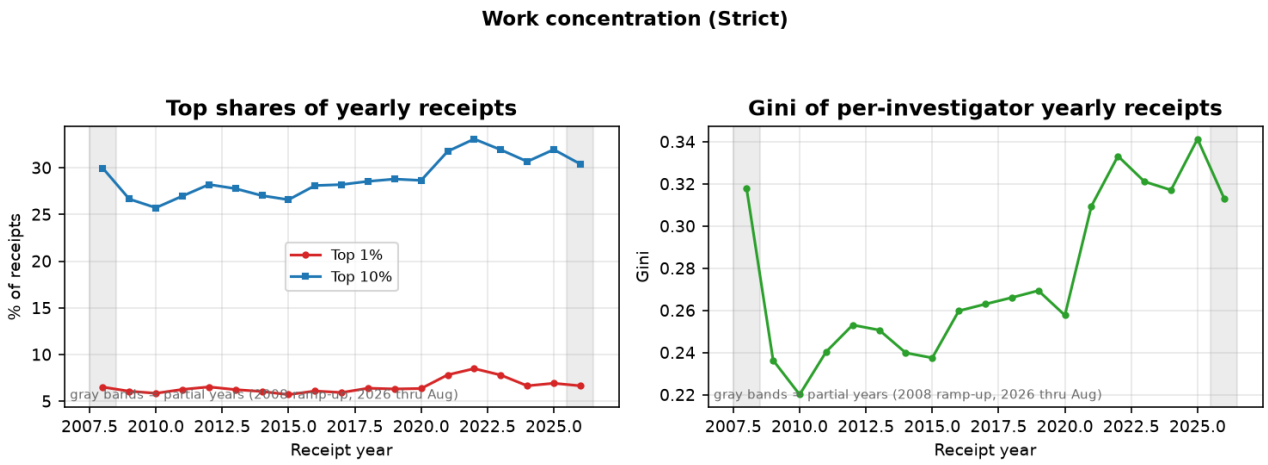


DEFINITIONS: Investigator vintage attrition: of the clinical investigators (CIs) first seen in a given year (their vintage), the share active again k years later. Right-censored (recent vintages show fewer years); 2008 excluded (file ramp-up distorts the base). Strict person: rows sharing identical normalized last/first/middle name + site + street + city + state + ZIP + country (exact-duplicate rows collapsed first). Conservative: one human at two address spellings counts twice.

- ≈19% of the 2009 vintage is active a year later; ≈9% a decade later.
- Curves look alike across vintages: most attrition is immediate.

Vintage = CIs first seen in that year. Right-censored; 2008 excluded.

Work is concentrating

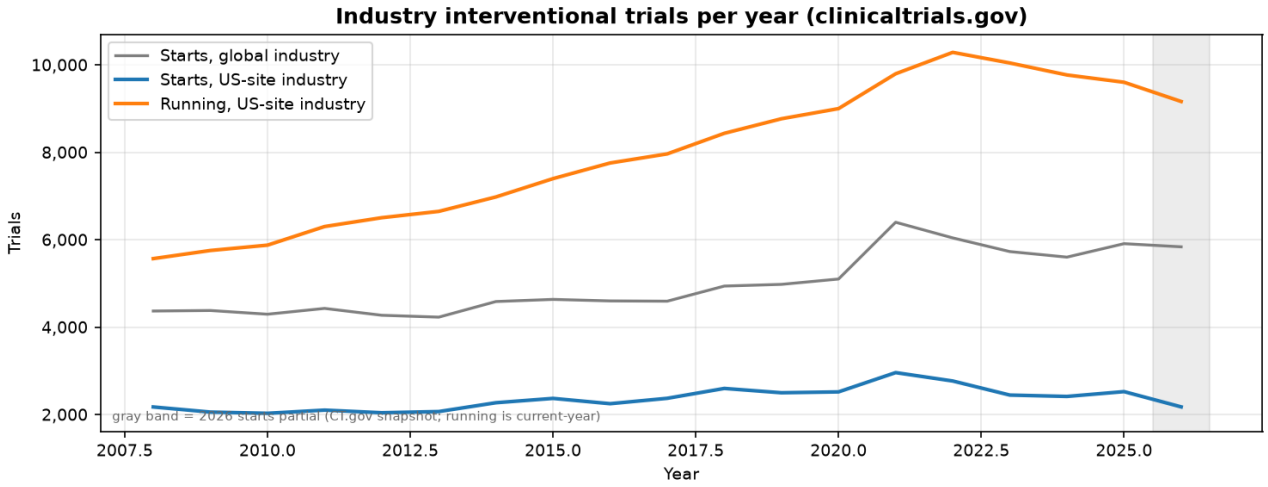


DEFINITIONS: Annual concentration: share of each year's receipts going to the year's top 1%/10% investigators by receipts, plus the Gini of per-investigator yearly receipts. Higher means work concentrated in fewer hands.

- Top-10% receipt share: 27% → 32%.
- Gini of yearly receipts: 0.24 → 0.34.

Annual within-year concentration among actives.

Demand: industry trial starts

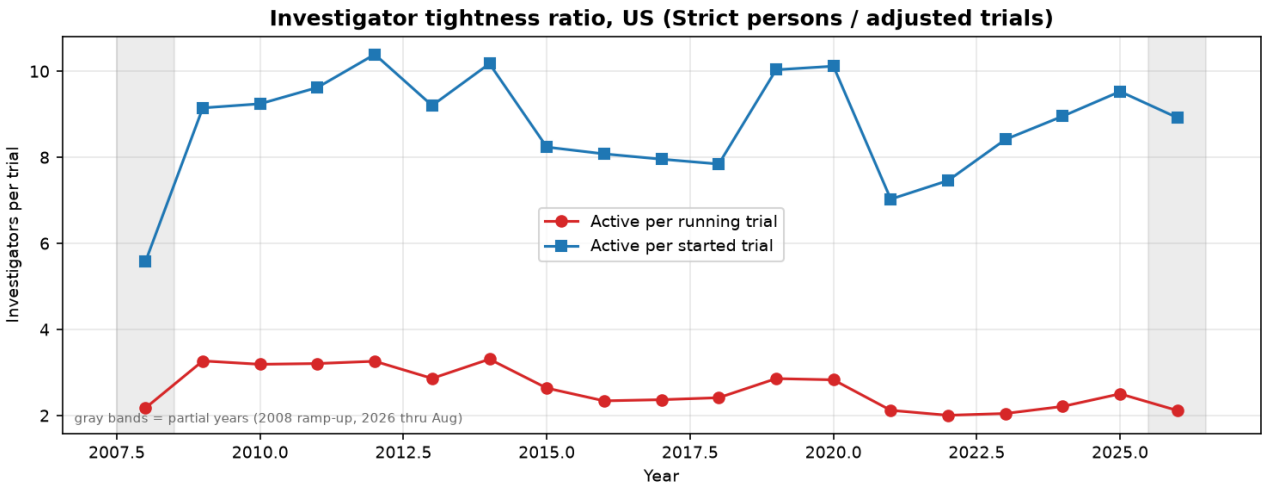


DEFINITIONS: Unadjusted benchmark: all TYPE=CI BMIS persons vs. all global industry-led interventional trial starts (clinicaltrials.gov). Adjusted benchmark: US-band BMIS persons vs. industry-led interventional trials with >=1 US site. Starts: trials by Study Start Date year. Running: start <= year-end AND (completion >= year-start OR still active with no completion date). CT.gov snapshot date recorded in manifest.

- Global industry starts: 5,911 in 2025.
- U.S.-site running trials: 9,604 in 2025.
- Starts and supply are not independent — ratios describe, not explain.

clinicaltrials.gov API v2, interventional + industry lead sponsor.

Tightness: investigators per running trial

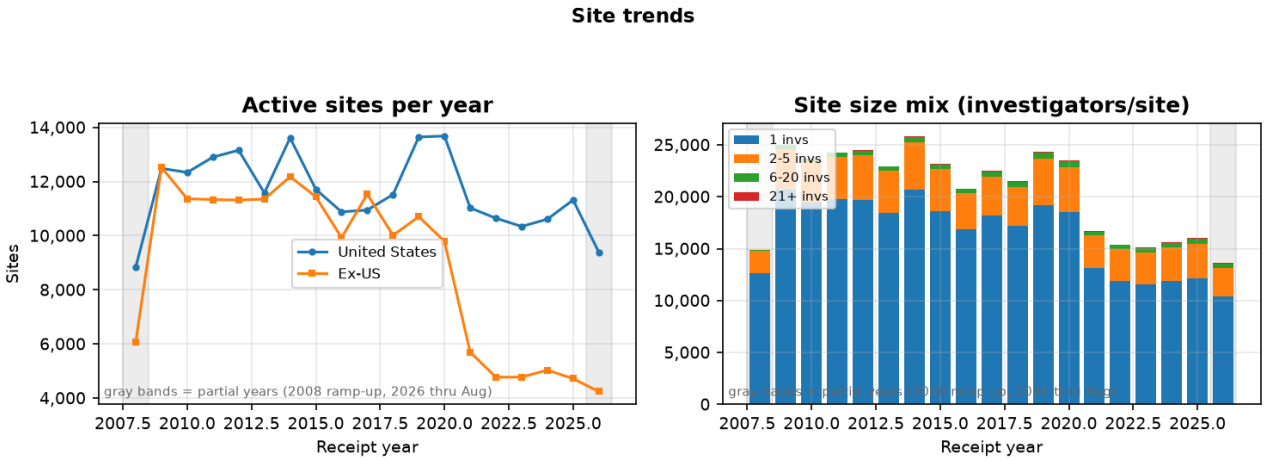


DEFINITIONS: Tightness ratio: active investigators per running trial. Descriptive workload proxy only; supply and trial counts are not independent (a supply crunch can suppress starts), so this is not causal. Active in year: person has >=1 BMIS receipt in that calendar year. New: first receipt ever falls in that year (2008-09 inflated: file starts Oct 2008). Returning: active and first seen earlier. Adjusted benchmark: US-band BMIS persons vs. industry-led interventional trials with >=1 US site.

- U.S. CIs per running trial: 3.3 → 2.5.
- Adjusted scope: U.S. investigators vs. U.S.-site trials.

Ecological ratio; supply constraints can move trial counts.

Sites: where the work happens



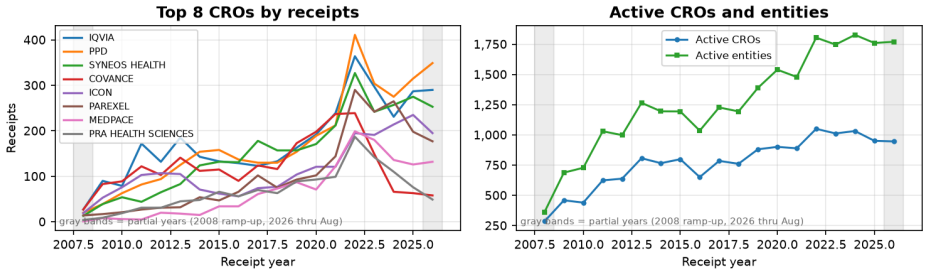
DEFINITIONS: A site is a PLACE (street address): Strict matches normalized street+city+state+country exactly (suite/floor markers dropped, ZIP excluded); Merged additionally unions same-house-number addresses with street-token Jaccard >= 0.6 within a city. Branches of one org at different addresses stay split. Site rollups use strict persons. 2008 (file ramp-up from Oct 1) and 2026 (file through Aug 31) are partial: shown faded, excluded from growth statistics.

- 16,036 active sites in 2025 (Strict addresses).
- Solo-investigator sites dominate; mega-centers are few.
- All-time leader: THE UNIVERSITY OF TEXAS MD ANDERSON CANCER CENTER (847 CIs, 5,390 refs).

Site = street address; branches of one org stay split.

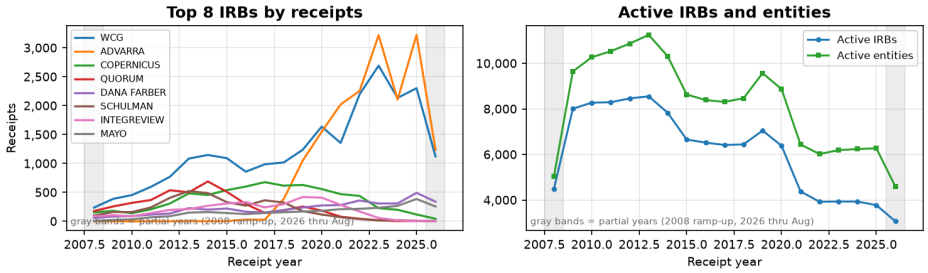
CROs and IRBs in context

CRO trends



DEFINITIONS: CRO views group BMIS TYPE=CRO rows by normalized company name: evidence-based aliases for known giants (PPD, Medpace, IQVIA, PRA, Syneos, ICON, Parexel, Covance, Celerion, others), else first site segment with suffixes/punctuation stripped. Same-name companies materially spanning states are state-qualified; aliases stay global. Residual variants stay split; cro_index lists variants. 2008 (file ramp-up from Oct 1) and 2026 (file through Aug 31) are partial: shown faded, excluded from growth statistics.

IRB trends



DEFINITIONS: IRB views group BMIS TYPE=IRB rows by normalized board name: evidence-based aliases for known boards (Advarra, WCG/Western, Copernicus, IntegReview, Schulman, Quorum, Sterling, Chesapeake), else first non-empty site segment with IRB stopwords, room markers, and numbers stripped. Same-name boards materially spanning states are state-qualified; aliases stay global. Residual variants stay split; irb_index lists variants. 2008 (file ramp-up from Oct 1) and 2026 (file through Aug 31) are partial: shown faded, excluded from growth statistics.

- 951 CROs and 3,779 IRBs active in 2025.
- All-time receipt leaders: PPD / WCG.

Normalized company/board groupings; residual variants audited.

Limitations & what’s next

- No therapeutic-area or phase fields in BMIS; rows carry date + site only.
- No person↔trial linkage; CT.gov cross-check is institution×city (experimental).
- Person/site counts are ranges across versions, not exact headcounts.
- Next: therapeutic-area mix via matched trials; global linkage; cross-site tracking.

Sources & reproducibility

- Supply: FDA BMIS quarterly file — https://www.fda.gov/drugs/drug-approvals-and-databases/bioresearch-monitoring-information-system-bmis
- Demand: clinicaltrials.gov Data API v2 — https://clinicaltrials.gov/data-api/api
- All code, methods, definitions: https://github.com/henryweimd/bmis1572
- Rebuild locally: uv run python pipeline/build.py